

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Study to Assess Deucravacitinib Safety in Pregnancy

Short Title/Acronym: Deucravacitinib Pregnancy Registry / OTIS Registry Study

Protocol Number: N/A (Registry/Observational)

NCT Number: NCT07017699

Trial Status: RECRUITING

Sponsor/Company Name: Bristol Myers Squibb

Investigational Product: Deucravacitinib (Trade Name: Sotyktu, Semantic Type: Clinical Drug, ATC Code: L04AA56, oral TYK2 inhibitor)

Phase of Development: Not Applicable

Medical Monitor: Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess pregnancy and infant outcomes among pregnant participants enrolled in an established North American pregnancy registry (Organization of Teratology Information Specialists [OTIS]) who were exposed to deucravacitinib. **Secondary Objectives:** To monitor the incidence of major congenital malformations (MCMs), spontaneous abortions, and other adverse pregnancy outcomes in women exposed to deucravacitinib during pregnancy.

2.2 Background & Rationale To evaluate the maternal, fetal, and infant safety of deucravacitinib exposure during pregnancy in women with Psoriasis (PsO). This study addresses the existing gap in real-world clinical safety data regarding the use of oral TYK2 inhibitors during gestation by gathering epidemiological data compared to unexposed and disease-matched cohorts.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Prospective Cohort) **Control Type:** Unexposed Cohort and/or Other Systemic Treatment Cohort (Pregnant participants without psoriasis/ autoimmune diseases, and participants with PsO on other systemic treatments) **Blinding:** Open-label **Randomization:** N/A (Observational)

3.2 Planned Enrollment & Duration Target \$N\$: 900 participants expected **Number of Sites:** Multiple sites in North America (including University of California San Diego / OTIS network)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Cohort 1: Deucravacitinib-exposed cohort

1. Currently pregnant during the enrollment period. 2. Diagnosed with psoriasis (PsO) validated by medical records when possible. 3. Exposure to deucravacitinib for any number of days, at any dose, and at any time from 2 days prior to date of conception (DOC) to the end of pregnancy. 4. Agree to the conditions and requirements of the study including the interview schedule, release of medical records, and the dysmorphology examination of live born infants.

Cohort 2: PsO Disease-matched unexposed comparator cohort

1. Currently pregnant during the enrollment period. 2. Diagnosed with PsO validated by medical records when possible. 3. May be exposed to systemic treatments for PsO. 4. Agree to the conditions and requirements of the study including the interview schedule, release of medical records, and the dysmorphology examination of live born infants.

Cohort 3: Non-disease unexposed comparator cohort 1. Currently pregnant during the enrollment period. 2. Agree to the conditions and requirements of the study including the interview schedule, release of medical records, and the dysmorphology examination of live born infants.

4.2 Exclusion Criteria Cohort 1: Deucravacitinib-exposed cohort

1. Pregnant women who have enrolled in this cohort study with a previous pregnancy. 2. Pregnant women who have used deucravacitinib for an indication other than PsO. 3. Women who do not have exposure to deucravacitinib anytime from 2 days prior to DOC to the end of pregnancy. 4. Women who have exposure to another oral tyrosine kinase 2 (TYK2) inhibitor or any oral Janus kinase (JAK) inhibitor from within 5 half-lives of DOC to the end of pregnancy. 5. Women who have exposure to methotrexate or an oral retinoid. 6. Retrospective enrollment after the outcome of pregnancy is known (ie, the pregnancy has ended prior to enrollment). 7. Results of a diagnostic test are positive for a major congenital malformation(s) (MCM) prior to enrollment. However, women who have had any normal or abnormal prenatal screening or diagnostic test prior to enrollment are eligible as long as the test result does not indicate an MCM.

Cohort 2: Disease-matched unexposed comparator cohort 1. Pregnant women who have enrolled in this cohort study with a previous pregnancy. 2. Women who have exposure to deucravacitinib or any other oral TYK2 inhibitor except deucravacitinib, or any oral JAK inhibitor from within 5 half-lives of DOC to the end of pregnancy. 3. Women who have exposure to methotrexate or an oral

retinoid. 4. Retrospective enrollment after the outcome of pregnancy is known (ie, the pregnancy has ended prior to enrollment). 5. Results of a diagnostic test are positive for an MCM prior to enrollment. However, women who have had any normal or abnormal prenatal screening or diagnostic test prior to enrollment are eligible as long as the test result does not indicate an MCM.

Cohort 3: Non-disease unexposed comparator cohort 1. Pregnant women who have enrolled in this cohort study with a previous pregnancy. 2. Women who have had exposure to deucravacitinib or any other oral TYK2 inhibitor, or any oral JAK inhibitor from within 5 half-lives of DOC to the end of pregnancy. 3. Retrospective enrollment after the outcome of pregnancy is known (ie, the pregnancy has ended prior to enrollment). 4. Results of a diagnostic test are positive for an MCM prior to enrollment. However, women who have had any normal or abnormal prenatal screening or diagnostic test prior to enrollment are eligible as long as the test result does not indicate an MCM. 5. Women exposed to a known, possible, or suspected human teratogen during pregnancy as confirmed by the OTIS Research Center. 6. Women who are diagnosed with PsO, or any other autoimmune disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Standard prescribed dose (according to product label). **Route of Administration:** Oral **Duration of Treatment:** Varied exposure (from 2 days prior to conception through the end of pregnancy based on real-world use).

5.2 Concomitant & Prohibited Therapy Permitted: Standard prenatal care treatments and allowed alternative systemic or topical treatments for PsO (for comparator cohorts). **Prohibited:** Methotrexate, oral retinoids, other oral TYK2 inhibitors, oral JAK inhibitors, and exposure to known, possible, or suspected human teratogens during pregnancy.

6. Study Timeline & Visit Schedule

Visit ID	Window	Key Activities/Procedures
Screening	Enrollment (Trimester 1-3)	Informed Consent, verification of PsO diagnosis and pregnancy status
Baseline	Day 0	Cohort assignment, exposure documentation, initial maternal health assessment
Interim	Throughout Pregnancy	Periodic maternal interviews, ongoing pregnancy monitoring, medical record release
End of Study	Post-delivery	Assessment of pregnancy outcome, dysmorphology examination of live-born infants

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of major congenital malformations among infants. **Measurement Method:** Evaluated via medical records review, maternal interviews, and physical dysmorphology examinations of live-born infants.

7.2 Secondary & Exploratory Endpoints Definition: Number of other pregnancy and infant outcomes. Incidence of minor congenital anomalies, infant developmental status, and maternal adverse events/complications during the gestational period.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection via periodic maternal interview schedule and medical records release. **Serious Adverse Events (SAEs):** Tracking of severe pregnancy complications, birth defects, or infant mortalities. **Data Monitoring Committee (DMC):** OTIS Research Center monitoring continuous safety surveillance for registry patients.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety/Registry Set (All enrolled pregnant women with confirmed exposure, plus unexposed and disease-matched control cohorts). **Sample Size Justification:** $N=900$ appropriately powered to detect meaningful epidemiological differences in major congenital malformation rates compared to the general population. **Handling of Missing Data:** Sensitivity analyses and appropriate data imputation models for missing or incomplete medical record validations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and patient privacy regulations. **Monitoring Plan:** Continuous monitoring of registry interview schedules, birth records, and outcomes across the OTIS network. **Audit Trail:** Electronic data capture tracking medical records release forms and expert dysmorphology examination reports.

11. Study Identifiers & Governance

Posting ID: 97371614327775341 **Primary ID:** NCT07017699 **Registry Number:** NCT07017699 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study to Assess Deucravacitinib Safety in Pregnancy **Official Regulatory Title:** The Deucravacitinib

Pregnancy Exposure Study: A Prospective Observational Study of Deucravacitinib Safety in Pregnancy **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/sotyktu-epar-product-information_en.pdf

12. Clinical Framework (Psoriasis & Pregnancy Specific)

Disease Area / Primary Condition: Psoriasis (PsO) **Preferred UMLS Name:** Drugs, Pregnancy Associated Treatment (**Preferred Name**): tazarotene 0.5 MG/ML Medicated Shampoo **MeSH Code:** D011634 **SNOMED ID:** 1963000 **SNOMED Hierarchy:** 100800008 | Psoriasis (disorder) | Psoriasis (disorder) **Diagnosis Threshold:** Verified medical diagnosis of PsO and confirmed pregnancy **Medical Methodology:** Organization of Teratology Information Specialists (OTIS) Pregnancy Registry Tracking **Optimization Target:** Maternal and infant safety monitoring during deucravacitinib exposure

13. Study Procedures & Methodology

Management Pathway: Prospective cohort observational tracking **Education Protocol (HCP):** Site-level registry reporting guidelines and clinical documentation **Education Protocol (Patient):** Registry interview schedule and informed consent parameters **Quality Audit Mechanism:** Medical record validation and dysmorphology examination by pediatric experts

14. Observation & Follow-Up Schedule

Total Duration: From enrollment (during pregnancy) until postpartum follow-up **Onsite Visit Cadence:** Dysmorphology examination of live-born infants postpartum **Remote Monitoring Frequency:** Periodic maternal interviews via OTIS throughout pregnancy **Checklist Review Interval:** Trimester-based registry updates

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				